

AI FOR BUSINESS TRACK · ROUND 2

Pavo Cloud

Autonomous Prior Authorization

The insurer's answer in 88.6 milliseconds. The whole round trip in under five minutes, not three to fourteen days.

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Prior Authorization Is Broken

3-14 days

to a decision

94%

of doctors report delays

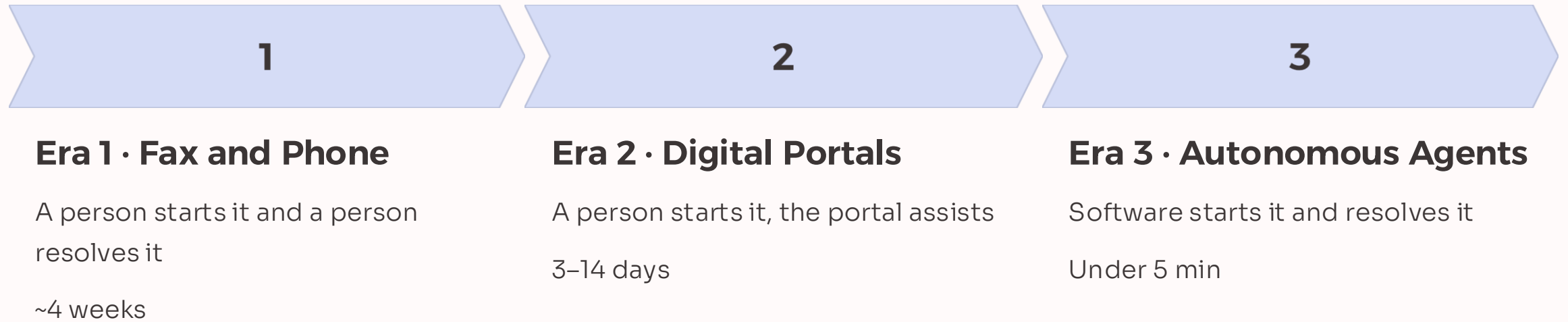
97%

of requests approved

\$35B

spent on the process each year

The Three Eras of Prior Authorization



Era 3: Agent to Agent

SOLUTION



Submitted Automatically

Clinical documentation leaves the EHR directly. No staff touchpoint, no portal login.



Unclear Cases Escalate, Assembled

A human reviewer opens a file already complete: records, rule, signature check, confidence score.



Clear Cases Close in Milliseconds

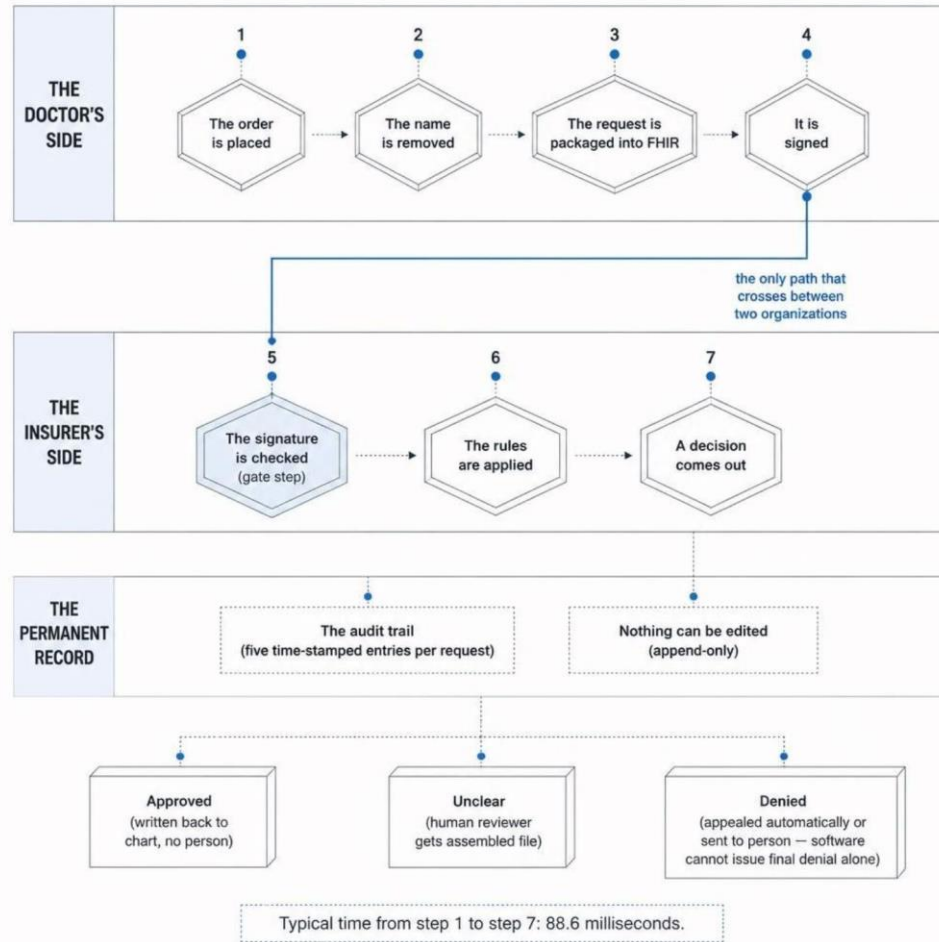
The full decision path takes 88.6ms, and 84.0 of those are the two RSA-2048 signatures every authorization writes. The deciding itself is the cheap part.



The Doctor Stays in Charge

Medical necessity remains a physician's call. Only the administrative burden disappears.

System Architecture



How It Works -

Stack: FastAPI · Next.js 14 · Supabase-ready · FHIR R4 · RSA-2048 signing · ZK circuits (circom/snarkjs) · timestamped audit log

Speed: Step 1 to step 7 in 88.6ms (median, n=200)

TRACTION

Customer Discovery & Validation

12+

interviews conducted with hospital administrators and clinical staff

8

insurance provider conversations completed

3

test surveys distributed to prior auth teams

2

letters of interest received from health systems

100%

of interviewed providers cited PA delays as a top operational pain point

The Safe Outcome Is the Default

Fixed Rules – Decides

Same input, same answer, every time

- Identity checks
- Coverage rules
- Price arithmetic
- Denial-reason sorting

AI Models – Assist Only

Every result carries a score. Below a threshold → a person

- Reading insurance cards
- Mapping plain English to a medical code
- Drafting appeal letters

Unmatched request → human reviewer, within 4 hours

Appeal below 70% confidence → held for a person

AI provider offline → authorization is unaffected

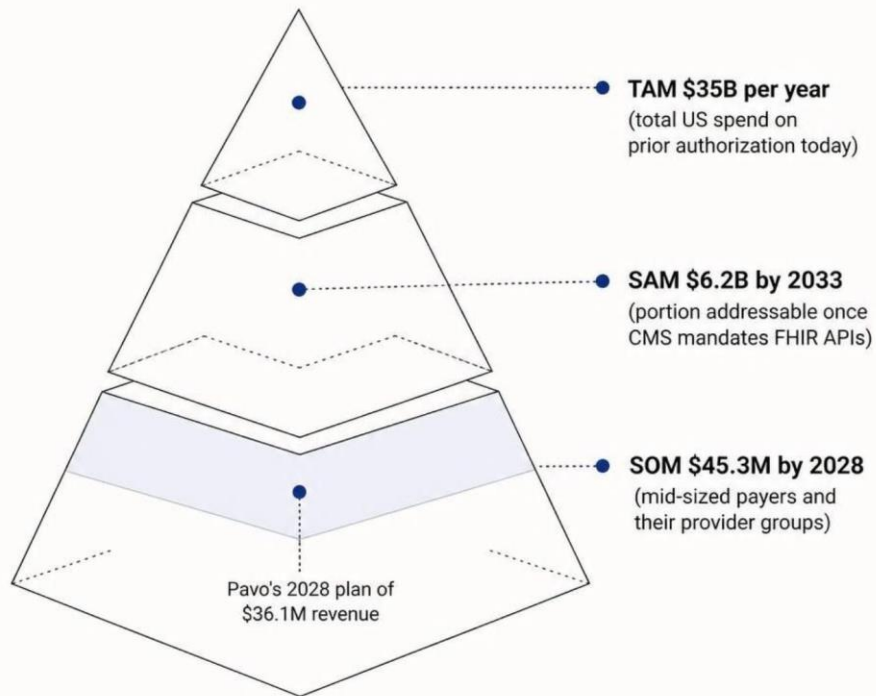
What We Have Not Solved

Gap	What it needs
The privacy proof reveals which covered condition a patient has	A more advanced circuit. Scoped for the next phase.
The proof's setup was done on one machine	A proper multi-party ceremony before any real use.
Provider identity is trusted, not verified	One integration against the federal provider registry.
Five coverage rules, not a rule library	Clinical staff encoding each insurer's criteria: our largest cost, and the real barrier to a competitor.

Everyone Else Still Waits for a Person

Capability	Pavo Cloud	Cohere Health	Availity AuthAI
Software can start a request with no human	✓	—	—
Decision without a human in the loop for clear cases	✓	—	—
Cryptographic sender verification	✓	partial	partial
Zero-knowledge patient privacy proof	✓	—	—

A Market a Regulation Is About to Create



Why Now

Federal mandate: CMS-0057-F requires FHIR-based prior-auth APIs by January 1, 2027, every payer has a hard deadline.

AI inflection: Agents now need a rail to initiate authorizations. Pavo is that rail.

Scale already exists: 47M payer-provider interactions a year still move through portals and fax.

BUSINESS MODEL

How Pavo Makes Money

\$3.00

per request, to the insurer

\$400

per practice, per month

\$20,000

per insurer, per year

\$36.1M

2028 revenue, from \$1.0M in 2026

87%

gross margin by 2028, 31% in year one

What a Customer Is Worth

\$6,699

to win one customer

\$57,533

back over three years

\$8.60

returned per \$1 spent

4 months

to earn it back

THE ASK

We Are Asking For \$8.0 Million

Rule library

clinical staff encoding each insurer's criteria

Certification

no insurer signs without it

The known gaps

provider registry, ceremony, rule library

\$5.58M

deepest point, late 2027

12 months

of cushion past breaking even